Preface

Patients with borderline personality disorder (BPD) constitute about 20% of inpatient and outpatient clinic patients. Yet in an era when costs of health care are soaring, the care provided to them is extremely inconsistent in quality and, worse, can easily be harmful. Borderline patients have a right to expect better or, at the very least, to expect that the mental health professionals responsible for their care have received basic training.

Regrettably, the training of most professionals is provided solely by their individual supervisors, who themselves have had no BPD-specific training. Training programs typically include little didactic information about BPD's psychopathology, let alone about evidence-based practices. Most professionals will readily admit that their training was insufficient for them to feel comfortable about, and capable of, treating BPD. To some extent, this lack of training and this lack of confidence are symptomatic of a collective countertransference within the mental health and medical fields; most psychiatrists and other physicians don't like and actively avoid borderline patients ("Borderline Personality: The Disorder That Doctors Fear Most," *Time* magazine cover story, January 19, 2009 [Cloud 2009]; Shanks et al. 2011). Of course, it is not just countertransference. In part, the negative attitudes about treating these patients are a result of persisting myths about what such treatments are thought to require (see table, overleaf). Unfortunately, also looming behind the lack of training, enthusiasm, and self-confidence by treaters is the reality that borderline patients can be difficult. Borderline patients actively challenge the hard-earned authority and the skill set professionals have been proud to attain.

Although these perspectives can help explain our profession's persisting failure to provide basic training, they do not justify it. It is time for mental health professionals to embrace the challenge posed by these patients. Borderline personality disorder will not allow us to ignore them. We have the knowledge to master the challenge and to be rewarded for having done so.

The purpose of this handbook is to provide the means through which psychiatrists and other mental health professionals can become "good

Myths about treatment of borderline personality disorder (BPD)

Myth	Clarification
BPD patients resist treatment.	Most actively seek relief from subjective pain; treatment for their personality disorder requires psychoeducation by clinicians.
BPD patients angrily attack their treaters.	Excessive anger and fearful wariness toward others, especially caregivers, are symptoms (i.e., instinctive transferences) of their disorder.
BPD patients rarely get better.	About 10% remit within 6 months, 25% by a year, and 50% by 2 years. Once patients have remitted, relapses are unusual.
BPD patients get better only if given extended, intensive treatment by experts.	Such treatment is required by only a subsample. Most do well within intermittent treatment by well-meaning nonexperts. Intense treatments can easily become regressive.
Recurrent risk of suicide invariably burdens treaters with serious liability risks.	Excessive burden or fears of litigation are symptoms of inexperience and of treatments that are poorly structured.
Recurrent crises require treaters to be available 24/7.	Such a requirement is rare and usually means that a different level or type of care is needed.

enough" to treat most cases of BPD competently and to take satisfaction from having done this well.

Splits are part of us all. Relationships with patients with BPD provide fertile soil for our internal splits to blossom. Only in retrospect have I recognized how my first reactions to these patients were so much governed by my negative fears of being controlled or seduced that those reactions had overwhelmed my positive hopes to protect and nurture. So I defensively went about the business of learning how to establish boundaries, recognize my borderline patients' covert anger, and prevent regressions. Of course, learning these lessons involved many mistakes, hurt feelings, and failed or, more commonly, abruptly shortened treatments. It also led to regrets, apologies, and humility. I grew self-conscious about my Calvinistic presence.

As my defensive fears of being intruded upon receded, my wishes to protect and be appreciated emerged. I worked at listening, validating, and being empathic, and in this process, I began to experience uninvited feelings of compassion and tenderness. I learned that it was an achievement for my BPD patients to be able to trust and depend on me. Clinicians who can contain defensive reactions toward their borderline patients and who can learn compassion toward them are well prepared to be helpful. The basic princi-

ples of Good Psychiatric Management (GPM) developed primarily from clinical experience and the personal growth that was required.

Treaters who practice GPM are encouraged to be active agents in helping borderline patients understand their inner experience, in reshaping their behavioral adaptations, and in establishing a good life. This treatment comfortably utilizes cognitive, behavioral, and psychodynamic interventions. As will be evident in this text, GPM borrows heavily from concepts introduced earlier by Winnicott (1953) such as "holding environment" and "good enough" parenting. These concepts decry too much specificity or perfectionism; they fit comfortably with GPM's emphasis on the borderline patient's interpersonal hypersensitivity (see Chapter 2 ["Overall Principles"]) and with a dyadic model of the therapy relationships.

Many years of trial-and-error experience forged this handbook's case management practices, which are practical and pragmatic. In retrospect, most of what I learned seems quite obvious. I hope that readers will find this to be true for them. I do not see myself as particularly gifted, and I do not believe that only experts can be effective treaters. I do believe that I have become "good enough" for most borderline patients and that if I can do this work, then so can most others.

John Gunderson

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Acknowledgments

As this book's primary author, I am responsible for whatever is speculative or proves misleading. In contrast, Paul Links, M.D., M.Sc., F.R.C.P.C., my long-standing colleague, is responsible for containing my excesses, for sharpening this book's messages, and for contributing his wise and practical clinical insights. Perhaps even more essentially, he can be held responsible for triggering the writing of this book. Had he not taken the initiative, invested his skills, and had the audacity to test, then confirm, the efficacy of a treatment called Good Psychiatric Management (GPM) (McMain et al. 2009) heavily modeled on what I had written, I would have been content to have my clinical perspectives remain the province of scholar and able supervisor. When GPM proved efficacious, it suggested that my clinical perspectives might offer a standard of care that should be more broadly taught. Hence this book.

Many others have contributed to the development of this book's case illustrations (Section III: GPM Workbook) and videos (Section IV: GPM Video Guide). Specifically, the following colleagues helped prepare the following case illustrations: Case 1—Brian Palmer, M.D.; Case 3—Brad Reich, M.D.; and Case 4—Lois Choi-Kain, M.D. Claire Brickell, M.D., suggested changes in all of the case illustrations and helped identify how they showed GPM's basic principles. The seemingly endless process of revising this book's contents was patiently overseen by my secretary, Linda DeVito Ghilardi.

The following colleagues helped prepare the videos: Video 2, "Diagnostic Disclosure"—Brian Palmer, M.D.; Video 5, "Managing Safety"—Paul Links, M.D., M.Sc., F.R.C.P.C., and Amy Gagliardi, M.D.; Video 6, "Managing Anger"—Claire Brickell, M.D.; and Video 8, "Managing Safety and Medications"—Paul Links, M.D., M.Sc., F.R.C.P.C. The video editing reflects the talents of Michael Williams from McLean's Materials Management Department. The patients were all "pseudopatients," artfully portrayed by members of McLean Hospital except for one from the Mayo Clinic (Video 2) and one

from the Department of Psychiatry, University of Western Ontario, London, Ontario (Video 8).

Since this handbook's initial draft, parts of it have been presented to many audiences. Each presentation has prompted revisions that have improved the book. Of particular note has been the opportunity of presenting each part of this book to colleagues at McLean Hospital. The breadth of their perspectives (from psychoanalytic to behavioral) and the depth of their clinical experience have forced me to reconsider, clarify, and sometimes omit sections that could not stand up to the test of their critiques. I am most seriously indebted to this rare collegial community for both stimulating and sharing what I have learned about treating borderline personality disorder.

J.G.

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SECTION I

Background

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CHAPTER 1

Introduction to Good Psychiatric Management (GPM)

This handbook is prescriptive: what to do, how to do it, and what not to do. It provides guides to turn to when one is unsure how to proceed with treatment. It recurrently encourages treaters to educate their patients about the disorder, most especially about the role of an inherited disposition and what is the expectable course. The interventions being recommended are meant to be commonsensical and simple to implement.

Section II, "GPM Manual: Treatment Guidelines," provides a condensed and clear description of the most essential and specific interventions in Good Psychiatric Management (GPM). It can be used as a guide for research from which measures of adherence and competence are available (Kolla et al. 2009). But, unlike a true manual, this handbook was not primarily written to ensure adherence so that efficacy can be tested. It does not proscribe "off-model" interventions; it recognizes that good clinical care is inherently flexible, pragmatic, and adapted to each patient. Its primary purpose is to describe clinical practices that clinicians can learn from and use in everyday practice. To facilitate this, Section III, "GPM Workbook: Case Illustrations," offers case vignettes interrupted by "decision points." At each of these decision points, different interventions are proposed and discussed. To further facilitate learning, Section IV, "GPM Video Guide: Demonstrations of the Approach," describes a set of nine videotaped interactions that can be viewed online. Here readers can see in vivo illustrations of the GPM model in practice.

Impelling this handbook's preparation was the vision that with adequate training, most psychiatrists and other mental health professionals can become "good enough" to treat most cases of BPD competently and to take satisfaction in a job well done. You don't need to be a specialist, to be self-

lessly devoted, or to have a larger-than-life personality to be "good enough"; you do need to be warm, reliable, interested, and unintimidated. and you need basic knowledge about case management. That is where this handbook comes in; if you know enough to avoid being harmful, you can be, surprisingly, very helpful. Such knowledge has typically been acquired by several years of work on inpatient services—a setting that provides unique opportunities to witness how borderline patients' behavior and moods are exquisitely sensitive to their social context and, most notably, how reactive they are to interpersonal events. This book is intended to condense and expedite such learning.

This handbook is designed to be a basic text for all hospital, outpatient clinic, or office-based psychiatrists or mental health professionals who need to assume primary responsibility for the treatment of BPD. It is particularly apt for psychiatric residents. GPM is a pragmatic, therapeutically effective approach emphasizing case management. It should not be considered as another form of individual psychotherapy—however, like good psychotherapies, it is guided by an understanding of the psychology of borderline psychopathology. More than most psychotherapies, it embraces neurobiological (genetic and pharmacotherapeutic) and social situational (stressors, family environment, and vocational) perspectives. It also embraces the perspective of positive psychology: GPM encourages patients to pursue satisfactory and meaningful lives. Because a large proportion of borderline patients are first seen by physicians within primary care and emergency department settings, the dos and don'ts of prescribing medications offer lessons that can prevent much burden and misuse. Some sections will be especially relevant to clinicians who do individual psychotherapy. Others provide the basic what-you-need-to-know for those who specialize in substance abuse, medication-resistant depression, and consultation-liaison psychiatry.

GPM's Place in Treatment Planning

GPM is "good enough" for most borderline patients. It can be used to expedite attaining competence and satisfaction from treating BPD. It is not meant to guide long-term outpatient psychotherapy. Short-term, intermittent, and nonintensive therapies are normative and are usually "good enough" (Bender et al. 2006). Within this approach, life's lessons can become a great ally in bringing about change if they become integrated. Treaters facilitate learning these lessons.

GPM is not meant to replace or compete with evidence-based therapies such as Dialectical Behavior Therapy (DBT; Linehan 1993), Mentalization-